



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نجاه عامر خلف الله حسانين

العنوان : القليوبية

الحي / المدينة : شبرا الخيمة

المحافظة : القليوبية

تاريخ تسجيل الإستمارة : 21-09-2026

رقم التليفون : 01117224803

رقم البطاقة : 28705272600201

العمر (تاريخ الميلاد) : 27-05-1987 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني الوظيفة: التوقيع:
الاسم: د/ندى حسن الوظيفة: التوقيع:
الاسم: د/نرمين رمضان الوظيفة: التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نجاه عامر خلف الله حسانين National ID: 28705272600201	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

Ans
Cubdo



MDT FORM

20/10/2028 G

Name	نبیاء عامر خلف الله حسنین	MRN	3302/24
Age	38 ✓	ID	CAV10CVC7...C1
Menopausal status		Phone	0117224803
NRS SCORE		Marital status	01095792486
BMI		Family history	-ve
CO morbidity		REASON OF MDT	

Clinical Data

Clinical Data				
BX: IDC II ER: 8/8 PR: 8/8 HER2: 0 Ki67: 15% Sonomammography: Mets Workup: CT CAP 7/2023 $\Rightarrow$ sclerotic bony lesions.				R Ps 0.1 L  TNM
Recommendations: Denovometastatic breast cancer to bone relieved *femara 11/2024, didn't relieve Painful Rth (normal) *2meta/3 nodes. Bone SGN 9/2025 $\Rightarrow$ Progressive Gurse by newly developed deposits. CDK4/6 / Fasbider + 2473 up				
MDT OUTCOME & PLAN Off to Fasbider / Torona @ 20meta 9.3.25				

SIGNATURE

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
					

Patient Name : الأستاذة / نجاه عامر خلف الله حسانين	Request No. : 13008329
Patient No. : 28705272600201	Request Date : 07/09/2024
Path No. : 3302/24	Verification Date : 24/09/2024
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 37 Y	Department : Out Patient
Referring Doc :	Registered by : Ali mostafa

Pathology Report

Clinical Data: Right breast swelling.

Nature of Specimen: Tru-cut biopsy, collected at 4/09/2024.

Gross Examination:

- Two grayish white tissue cores up to 2 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-8).

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+2+1) Grade II
- * Ductal carcinoma in situ: No.
- * Perineural invasion: No.
- * Angiolinvasion: No.
- * IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki67: 15% nuclear reaction of tumor cells.

Diagnosis:

Right breast swelling, Tru-cut biopsy:

Invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).

Verified by :

Dr. Sagda El-Kady

فنيح علي
ام نعتي
9/9

Printing Date : 24/09/2024 09:44:36

Page 1 of 2

Printing By : marwa mohamed youss

CAP
ACCREDITED

شرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name : الأستاذة / نجاة عامر خلف الله حسنين

Patient No. : 28705272600201

Path No. : 3302/24

Sex : Female

Age : 37 Y

Referring Doc :

Request No. : 13008329

Request Date : 07/09/2024

Verification Date : 24/09/2024

Financial Category : اوزام دار السلام - ميلاده

Department : Out Patient

Registered by : Ali mostafa

Pathology Report

- I067: Positive in 15% of tumor cells.

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Rana Fathy.

Dr. Sagda Elkady.

Verified by :

Dr. Sagda El-Kady

Printing Date : 24/09/2024 09:44:36

Page 2 of 2

Printing By : marwa mohamed youss





Patient Name: Nagat Amer Khalaf Allah Hasanain	Date : 03/09/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 37 Year(s)
Medical ID: 28705272600201	Pat. ID: 78486

**Full Field Digital bilateral breast mammography & Complementary
Ultrasound**

Indication:

Right breast palpable lump

Report:

- Heterogenous fibro glandular density may obscure small lesions (ACR C)

Right breast

- UIQ irregular hyperdense ill-defined spiculated mass lesion measuring about 50x50mm. Complementary US showing UIQ irregular hypoechoic mass lesion with spiculated margins and peripheral desmoplastic reaction. It is located at 1-2O'clock reaching the nipple with nipple retraction, the lesion is seen reaching the skin.
- Indeterminate enlarged level I axillary lymph node measures 10 mm in TS diameter with 2.7mm cortical thickening
- No level II & III axillary or supraclavicular Lymphadenopathy

Left breast

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- No solid or cystic lesions
- No dilated ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Right breast malignant mass with indeterminate level I axillary lymph node (BIRADS 5) for histopathological correlation
- Normal mammographic study of left breast (BIRADS 1)

Best regards,

Dr Reem Mahmoud
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCC, Cairo University

نجات
أمر
خالد
الله
حسنين

3/9/2024

Patient Name: Nagat Amer Khalaf Allah Hasanain	Date : 29/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 38 Year(s)
Medical ID: 3270/2024	Pat. ID: 78486

POST

CONTRAST MULTI-SLICE CT SCAN OF THE CHEST , ABDOMEN AND PELVIS

Reported case of breast cancer

- Right breast mass measuring 4.3 x1.9 cm
- Multiple bilateral enlarged axillary L. Ns ,the largest at right axilla 15 x10 mm..
- Single subcentimetric pulmonary calcified nodule at right lower lobe 6mm likely post granulomatous disease.
- Clear both lung fields.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average size cardiac size. No pericardial effusion detected.
- Normal appearance of the chest wall.

II Abdomen & pelvis:

- The liver is average size in size with homogenous CT texture. No focal lesions.
- Average size gall bladder with no gall stones.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Few millimetric paraaortic L. Ns.
- Normal appearance of the pancreas, and both suprarenals.
- Normal appearance of the other visualized pelvic organs.
- No abdominal or pelvic masses or collections.
- IUCD in place .

II.Scanned skeleton:

- sclerotic bony lesions of L2 and to less extent L2 and D12.

Opinion:

نرجاء على مقارنة

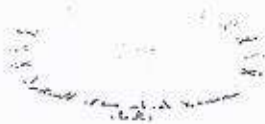
Patient Name: Nagat Amer Khalaf Allah Hasanain	Date : 29/07/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 38 Year(s)
Medical ID: 3270/2024	Pat. ID: 78486

- *Right breast mass with axillary lymphadenopathy*
- *Multiple sclerotic lesions of D12 , L2 and L4 vertebrae likely metastatic for correlation with bone scan.*
- *No pulmonary or hepatic metastatic deposits*

DR/Reham Khedr,,MD

29/7/2025

Not Approved



Preventive Program for Women's Health
Ministry of Health and Population

البرنامج الوقائي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : نجاة عابر خلف الله حسانين

Sex:

MDT form
3- Medical number:
Female

83333743

Old Med :

Old Med :

2-ID number:

3270-2024

Patient Age : 37 Y



Family history

Family history

Phone

Phone

BMI

BMI

marital status

Marital Status

HRS Score

HRS Score

GROUP

GROUP

Co morbidity

Co morbidity

Menopausal
status

Menopausal status

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT

Date of MDT

Date of MDT

27/10/2024

Breast Imaging :

at 5x5 cm UIQ at 12 o'clock reaching skin

Clinical data

Mets Workup :

cap:multipia osseous lesion likely mets

BX IDC BX COMMENT: II	ER S/S ER COMMENT:	PR S/S PR COMMENT:	Her 2 Neu 0 Her 2 Neu COMMENT:	SISH SISH COMMENT:	Ki67 15
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T	N	M 1 (BONE)
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Sonomammography

PATIENT HISTORY

Rt breast deNOVO METS TO BONE with destructive L2 bony lesion

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

For Zoladex, AI , Zometa/3 month (Check E2 and PSH/3m),
and palliative RTM to L2

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د. رشاوي صبره [Signature]	RADIOLOGY	RADIOLOGY [Signature]
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition



Patient Name: Nagat Amer Khalaf Allah Hasanain	Date : 11/03/2025
Exam : C.T CHEST WITH CONTRAST	Age : 37 Year(s)
Medical ID: 3270/2024	Pat. ID: 78486

Post CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

✚ **Reported case of right breast cancer. Compared to last study dated 24-9-2024**

I Chest:-

- **Still noted** Prominent right axillary lymph nodes level I, largest still measuring about 1.3x0.8 cm.
- Still noted few Right lung tiny pulmonary nodules the largest seen at lobe apical segment of lower lung lobe calcified pulmonary nodule still measuring about 4 mm, likely granulomatous.
- No masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal pathological lymph nodes.
- No evidence of pleural effusion, thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.

II Abdomen & pelvis:

- Mildly enlarged liver with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- IUD is noted otherwise unremarkable appearance of the other pelvic organs.
- Multiple pelvic phleboli.
- No abdominal or pelvic masses or collections.

***Scanned skeleton:**

- More sclerotic texture of the mixed lytic and sclerotic lesion noted at L2 vertebra,.



Patient Name: Nagat Amer Khalaf Allah Hasanain	Date : 11/03/2025
Exam : C T CHEST WITH CONTRAST	Age : 37 Year(s)
Medical ID: 3270/2024	Pat. ID: 78486

- Nore sclerotic texture of mixed lytic and sclerotic bone lesion at D1
- More sclerotic teture of multiple osseous mixed lytic and sclerotic lesions noted at right iliac bone, both sacral ala, L4 D1 vertebra

Opinion:

Reported case of breast cancer showing;

- nore sclerotic texture of mutiple osseous lesions.
- Stationary course of the rest study.

Dr. Marwa Kamal Saber

Prof. Menna Tallah Hanafy, M.D.



NAME: NAGAH AMER KHALAF ALLAH HASANAIN
DEAR SIR

ID: 101820115
13/10/2024

Tc^{99m} MDP Whole Body Biphasic Bone Scan

Technique:

Whole body anterior and posterior blood-pool and delayed 3 hours scans were taken after IV injection of 20 mCi of Tc^{99m} MDP.

Findings:

- Multiple hot foci are seen in dorsal, lumbar vertebrae, right humerus and right sacroiliac region
- Rest of the skeleton is free
- No abnormal soft tissue uptake

Impression:

- Multiple scattered hot foci , likely metastatic

MUCH OBLIGED
PROF.DR. FATIEN KAMEL
Faculty of Medicine-Ain Shams University
AMR

Name : Najah Amer Khalaf Allah Hassanain
Branch : Dokki

Date : 17-09-2026 02:02
Medical No. : 463999

NUCLEAR MEDICINE BONE SCAN REPORT

Clinical Indication: 39-year-old female with right breast carcinoma. Referred for follow-up skeletal evaluation compared to prior study dated 29/09/2025.

Procedure: Whole-body planar bone scintigraphy, anterior and posterior views, following intravenous administration of 99mTc-MDP.

Findings:

- Compared to the prior study dated 29/09/2025, the previously documented active lesion at D1 demonstrates complete interval resolution with no definite abnormal uptake. Previously noted lumbar spine activity shows marked regression, leaving only residual faint uptake without discrete focal lesions. No abnormal osteoblastic tracer uptake is identified in the bilateral skeleton. A elsewhere in the surveyed axial or appendicular skeleton remains at the right small, persistent focus of low-grade uptake likely benign/odontogenic in sacroiliac articulation.
- Focal uptake at the left mandible is likely benign/odontogenic in origin.
- Mild, symmetrical periarticular tracer uptake involving major joints is consistent with age-related degenerative changes.
- Renal radiotracer excretion is prompt and symmetric.

Impression:

- Significant regression of skeletal metastases compared to September 29, 2025, with resolution at D1 and residual uptake marked reduction at the lumbar spine, and residual uptake limited to the right sacroiliac joint. Follow up is recommended.

Dr. Alaa Nagi MD Nuclear Medicine